

CONCEPT PAPER QUESTIONS

Write a short concept paper that explains your idea, following the structure listed below. This concept paper helps us understand:

- The root problem you are trying to solve
- Whether technology is actually needed to solve it
- How your idea would work in practice and what you hope to transform
- How you participated with your community and thought about risk, data, and the people affected.

We are not looking for polished tech jargon or finished products. We are looking for clear thinking, practical judgment, and careful choices. **Please make sure that your total word count doesn't exceed 1500 words.**

● Q1 > Stage 1: Your Objectives:(Hildah)

- **What problem are you trying to solve, and for whom?**

Describe the problem to be addressed and its effect on people and your community.

Be specific. Explain why this problem matters now.

Sexual Gender based violence(SGBV) remains a significant challenge affecting women and girls especially among those living in rural and peri-urban areas.According to Kenya demographic health survey (KDHS) 2022 ,34% of women aged (15-49) have experienced a form of gender based violence and these are the reported and documented cases. Survivors of sexual and gender-based violence often face significant barriers when seeking help, including stigma, fear of retaliation, limited awareness of available services, and restricted access to trusted support systems. These challenges discourage many from coming forward, resulting in delayed or no access to care, protection, and justice.

Many survivors do not know where to report cases or how to access essential services such as healthcare, legal aid, police support, or psychosocial counseling. In some cases, they are forced to travel long distances or rely on informal referrals and inaccurate information on GBV response which can delay critical care and compromise evidence

collection. Additionally, concerns about confidentiality and safety discourage survivors from coming forward.

This problem is especially critical now due to increasing awareness of SGBV but limited systems that support safe, confidential, and survivor-centered reporting and referral. While services may exist, they are often not well structured, poorly coordinated, or inaccessible to those who need them most.

Youth Changers Kenya has developed a functioning incident tracking system, now ready for pilot testing with community champions, to capture and manage SGBV cases.

Then explain: What you want to change; Why technology is necessary (if it is); What success would look like in real life; What you seek to transform.

YCK aims to improve access to timely, confidential, and accurate SGBV related information and referral pathways for SGBV survivors. We want to:

- Enable survivors to report cases safely and anonymously
- Provide clear, real-time guidance on where and how to seek help near them.
- Connect survivors to the nearest verified health facilities, police stations, legal aid providers, and counselors for support ,care and justice.
- Share correct SGBV information to survivors while respecting their autonomy and privacy .

Technology is necessary because the current system relies on physical access, word of mouth information and referrals and unstructured service delivery. An AI-powered tool can bridge these gaps by:

- Offering 24/7 access to information and necessary support without requiring physical presence.
- Providing instant, location-based referrals to services
- Ensuring privacy and confidentiality, which are critical for survivors
- Standardizing information to reduce misinformation and confusion
- Scaling support to reach more people, including those in remote areas

● Q2> Stage 2: Team composition

- **Who are your team members for this project?**
- **Who are you bringing onboard?**
- **How is the community involved? What is the stakeholder ecosystem?**

(In addition to your regular team; please include in details who you will be engaging with i.e researchers; psychologists; community leaders; topic experts you will consult with to evaluate the scope/usability of your tool).

Please note that numerous people are involved in the creation and operation of an AI system – and more than just people writing code! Your objective will fundamentally inform the team composition required to create the system fulfilling the objective. This includes the skills and expertise needed, but also the diversity of its members regarding backgrounds, perspectives, and experiences with the environment for which your system is developed.

The team will be made of different people with technical, ethical, and community-based expertise.

1. Project Lead: Who is responsible for coordinating the project, engaging stakeholders, and ensuring that the solution remains relevant to the needs of the survivors and realities of the communities. The project lead will also oversee the planning, implementation, and sustainability of the AI-powered system.
2. Technical Lead: Who will lead the development and design of the platform, including backend services, workflow engines and data security measures.
3. Monitoring and Evaluation Lead: who will lead the pilot evaluation, and impact assessment.
4. Program Officer: Who will participate in the co-designing of the platform, testing out offline data capture, identify gaps and ensure that the tool reflects how the incidents are documented in the community.

5. Community leaders (grassroots organizations, community gatekeepers, youth leaders) and human rights advocates: Will be engaged to review cultural appropriateness, accessibility concerns, language use and trust barriers.

The community will be engaged in this project from the design phase as co-creators, validators and accountability partners. They will also be engaged in testing the usability of the platform and will provide feedback that will better the platform. The community will also access the platform, the language used to ensure culturally appropriate, referral pathways, trust, and help-seeking behavior.

The stakeholder ecosystem for this project is broad because addressing sexual and gender based violence (SGBV) requires a multi-sectoral approach. Primary stakeholders include: survivors of SGBV, YCK champions, Community Mobilizer, Counselors, program lead and Executive Director. The secondary stakeholders include: SGBV service providers, grassroots organizations, legal aid providers, healthcare providers, and researchers.

🟡 Q3> Stage 3: The System

Describe the tool, system, or process in simple terms. This could be:

- A digital tool
- A data system
- A decision-support process
- A hybrid approach

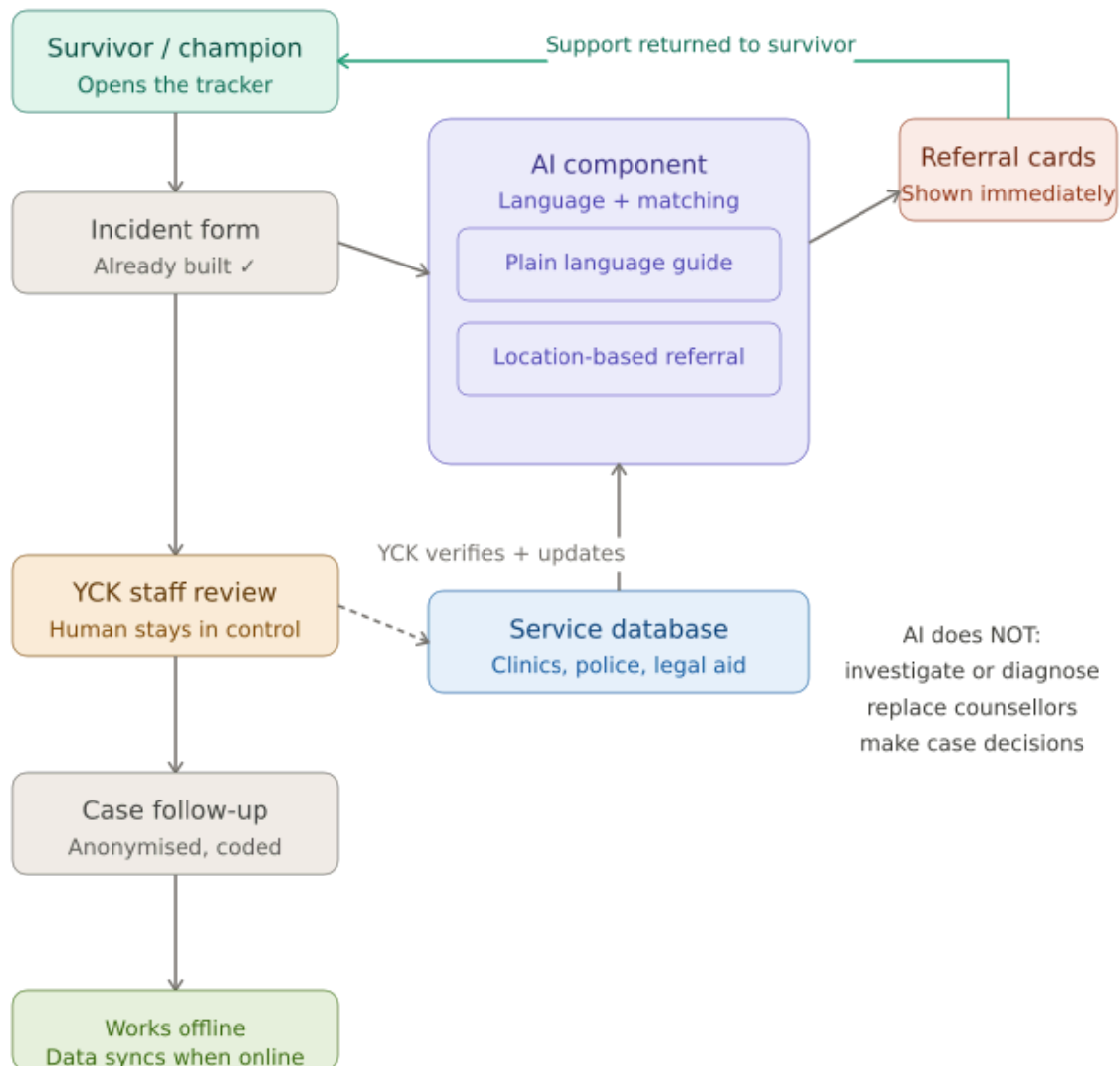
Explain: What the system will do; What it will not do: Where humans remain in control; If helpful, describe sketches, workflows, or prototypes, even informally.

Youth Changers Kenya has built and deployed a functioning incident tracking system, a progressive web app used by survivors and community champions to report SGBV incidents and by counsellors and staff to manage cases, trigger workflow notifications, and generate anonymised reports. The core infrastructure, case logging, referral management, offline sync, secure database, and staff notifications, is operational.

This funding enables the next phase: integrating an AI layer on top of this working system. The diagram below illustrates how the two components connect. Specifically, when an incident is submitted through the form, the AI component will: (1) match the

incident type and reported location to a verified database of nearby services, and (2) surface plain-language referral cards guiding the survivor to the most appropriate support, medical, legal, or psychosocial. The referral cards are returned immediately to the survivor or community champion at the point of reporting, ensuring support is accessible without delay.

[See system diagram below]



The AI responds to the form submission; it does not hold a conversation, investigate cases, or replace the role of counsellors and staff. Human oversight remains central, YCK staff review all cases, maintain the service database, and manage follow-up. The system is designed to work offline, with data syncing once connectivity is restored.

Q4> Stage 4: The Data

What types of data are needed?

The first data we need will be the survivors data. This will include ;

- Their nearby location to enable the system to suggest nearby services to access services.
- Information about the case to enable the system tailor information and support needed accordingly.
- Survivors or champions generate their own memorable reference code at the point of reporting — for example NT111974 using initials and birth date. An optional email field is available for confirmation only and is not linked to the case record.

The system will need services and referral data

This will include step by step referral pathway , information on SGBV like forms of GBV ,Nearby health facilities and police stations, verified contacts of counsellors and legal aid officers.

Who collects it and for what purpose?

For the user/survivors data, this will be self-reported by the survivor while reporting their case. The survivors at times will be supported by community champions in reporting and using the tool.

For the service data, including the referral pathways ,legal aid and counsellors will be collected by the YCK staff in collaboration with community stakeholders from health facilities, police stations and even county governments.

How will consent, privacy, and security be handled?

The user will be clearly informed on the system , what data is being collected and why before using the tool. Minimal personal information of the users will be collected unless necessary for the specific cases that will require follow up. The survivor's identity remains protected throughout — no names or identifying details are collected at any stage.

For security purposes,the tool will have only authorized personnel who can access it and the data will be encrypted and securely stored.

Reflect on:

- **Possible bias in the data**

Interior services that are not commonly known or popular may miss out on the service database Like interior health facilities.

At times the users who do have smartphones or access to internet do not represent the most vulnerable groups making it possible for survivors to be left out

- Historical or social harms linked to this data

Historically, data collected from SGBV survivors has been misused, shared without consent, or used in ways that re-traumatise survivors. This system is designed to prevent that — no identifying data is stored, cases are coded from the point of entry, and only authorised YCK personnel can access case details.

- How you will reduce harm rather than reproduce it
 - ❖ Clear communication for the user in terms of consent and information being collected
 - ❖ Trauma-informed language-the tool will use SGBV sensitive language for the users.
 - ❖ Allow users to access information without revealing any personal information that may identify them.
 - ❖ Community partnerships-work with local institutions like legal aid ,health facilities e.t.c.to ensure relevance of services

- ❖ Partner with trained community champions who will support survivors in using the tool to access information and report their cases.

You do not need perfect answers, but you do need honest ones.

🟡 Q5> Stage 5. Selecting and developing a model

- **Have you started thinking about what kind of tool or approach you might build, for example, something rule-based, a scoring system, or something that learns from data?**
- Why does this choice fit the context?
- What trade-offs are you making?

More complex technology is not automatically better.

Include a rough timeline and explain how community input will shape changes.

Youth Changers Kenya has developed a functioning incident tracking system — a Progressive Web App — that is ready for pilot testing and validation with community champions and survivors. The core infrastructure, including case logging, referral management, offline sync, secure database, and staff notifications, has been built. This funding enables us to develop and integrate the AI matching layer on top of this foundation, so that both components are tested and validated together.

The system uses a matching and retrieval approach — when an incident is submitted through the form, the AI matches the incident type and location to a verified database of nearby services and returns plain-language referral cards to the survivor or community champion. This approach fits the context because cases of violence are often underreported and survivors lack immediate access to accurate information about where to seek help. A simple, reliable matching system addresses this directly without introducing unnecessary complexity.

We deliberately chose this approach over a conversational chatbot. A chatbot introduces risk in a trauma-sensitive context — it can generate inconsistent or harmful responses and creates false expectations of human support. Our approach keeps the AI narrow and auditable: it matches structured inputs to a verified database, and humans remain responsible for all follow-up action.

The trade-off we are making is choosing simplicity and reliability over sophistication. A more complex system may offer richer interactions but introduces greater risk of failure, higher maintenance demands, and reduced trust from survivors and community champions. A system that works consistently and can be used offline is more valuable in our context than one that is technically advanced but fragile.

Community input will shape the system throughout. Champions and survivors will be involved in pilot testing, and their feedback on accuracy of referrals, ease of use, language appropriateness, and feelings of safety will directly inform iterations before wider rollout.

● Q6> Stage 6. Testing the model (Hildah)

- **How will you know if this works?**

We will know the tool is working if ;

- ❖ The users are easily able to use the system and get relevant support.
- ❖ The users are successfully connected to nearby services like health facilities, police stations, counsellors and pro-bono lawyers.
- ❖ Report and data from service providers like health facilities show an increase in access to SGBV related services
- ❖ Feedback from the users of confidentiality , safety and ease of using the tool.

- **How will you test the system?**

The testing system will occur in phases:

- ❖ The first group to test out will be the YCK team and pinpoint any gaps and technical challenges they would see.
- ❖ The next testing phase will be a selected group(pilot group) of SGBV survivors who will introduce real time experiences to the system and get real time informal and referral services.
- ❖ The system will be periodically improved depending on the user feedback.

- **Who will be involved in testing?**

- ❖ YCK team members including the technical team

- ❖ Selected group of GBV survivors
- **What feedback will matter most?**
 - ❖ Safety and confidentiality -do the users feel like their data is protected,their privacy is upheld and safety while using the tool
 - ❖ Relevant and accurate service referrals -are the users getting referrals in their contexts and locations.

Indicative timeline:

Month 1–2 — Compile and verify service database in partnership with local health facilities, police, and legal aid providers.

Month 2–3 — Develop and integrate the AI referral matching layer into the existing PWA.

Month 3–4 — Internal testing with YCK staff and counsellors, identify gaps.

Month 4–5 — Pilot with a selected group of community champions and survivors, collect feedback on accuracy, safety, and ease of use.

Month 5–6 — Iterate based on feedback, update service database, and prepare for wider rollout.